

1. Administrative & Study Identification

Full Study Title: Safety and Efficacy of Tisotumab Vedotin Monotherapy & in Combination With Other Cancer Agents in Subjects With Cervical Cancer **Official Title:** A Phase 1b/2 Open-Label Trial of Tisotumab Vedotin (HuMax®-TF-ADC) Monotherapy and in Combination With Other Agents in Subjects With Recurrent or Stage IVB Cervical Cancer **Short Title/Acronym:** innovaTV 205 / GOG-3024 / ENGOT-cx8 **Protocol Number:** 19-092 **NCT Number:** NCT03786081 **Posting ID:** 35140835394056246 **Trial Status:** ACTIVE_NOT_RECRUITING **Sponsor Name:** Pfizer **Investigational Product:** Tisotumab Vedotin (antibody-drug conjugate) in combination with bevacizumab, pembrolizumab, or carboplatin **Phase of Development:** Phase I/II (Phase 1b/2) **Medical Monitor:** Sponsor Medical Monitor / Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To determine the dose-limiting toxicities (DLTs), maximum tolerated dose (MTD), and recommended phase II dose (RP2D) of tisotumab vedotin (TV) in combination with other agents (Phase 1b), and to evaluate the Objective Response Rate (ORR) of the combinations (Phase 2) per RECIST v1.1. * **Secondary Objectives:** To evaluate the number of adverse events (AEs), duration of response (DOR) per RECIST v1.1, time to response (TTR), progression-free survival (PFS), overall survival (OS), and overall safety and tolerability profiles.

2.2 Background & Rationale Tissue factor is highly expressed in cervical carcinoma and can be effectively targeted by tisotumab vedotin (TV), an investigational antibody-drug conjugate. Patients with recurrent or metastatic cervical cancer (r/mCC) who progress on standard-of-care treatments have a poor prognosis and limited therapeutic options. This study aims to provide new therapeutic options by exploring TV both as a monotherapy and as part of novel combination regimens (with bevacizumab, pembrolizumab, or carboplatin) in earlier lines of treatment.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional (Dose-escalation and Dose-expansion) * **Control Type:** Parallel experimental cohorts (Single-arm per cohort, no comparator) * **Blinding:** Open-label * **Randomization:** N/A

3.2 Planned Enrollment & Duration * **Target N\$:** 214 evaluated participants * **Number of Sites:** Multicenter (United States and Europe, e.g., Belgium, Denmark, Italy, Netherlands, UK) * **Estimated Study Start:** Late 2018 * **Estimated Primary Completion:** Data cutoff reached in January 2022 for primary analysis

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Must have histologically confirmed squamous, adenosquamous, or adenocarcinoma of the cervix, presenting as recurrent or stage IVB disease. 2. Must have baseline measurable disease per RECIST v1.1. 3. Have an Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 4. Have progressed on or after standard of care treatments, or be ineligible/intolerant to standard of care (specific to treatment arms), or not have received prior systemic therapy for recurrent/stage IVB disease depending on the assigned arm.

4.2 Exclusion Criteria 1. Pregnant, breastfeeding, or expecting to conceive children within the projected duration of the trial and for at least 6 months after the last trial treatment administration (must agree to adequate contraception). 2. Ongoing or significant incapacity that interferes substantially with normal life functions. 3. Known history of active ocular surface disease at baseline or prior history of cicatricial conjunctivitis. 4. Clinically relevant bilateral hydronephrosis which cannot be alleviated by ureteral stents or percutaneous drainage. 5. Clinical signs or symptoms of gastrointestinal obstruction requiring parenteral hydration and/or nutrition. 6. Clinically significant bleeding issues or risks, including recent hemoptysis, gastrointestinal or vaginal bleeding requiring PRBC transfusion, or wound healing complications. 7. Clinically significant cardiac disease. 8. Requirement for anti-coagulation therapy (for specific arms).

5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** Recommended Phase 2 Dose (RP2D) established as TV 2 mg/kg plus either bevacizumab 15 mg/kg, pembrolizumab 200 mg, or carboplatin (Paraplatin) AUC 5 administered on Day 1 once every 3 weeks. Certain expansion arms evaluate alternative regimens (e.g., weekly TV dosing on a 28-day cycle). * **Route of Administration:** Intravenous (IV) infusion * **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive care for managing infusion reactions, nausea, or routine co-morbidities. * **Prohibited:** Any other concurrent systemic antineoplastic therapy or investigational agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Scans (RECIST 1.1), ECOG Status
Baseline	Day 0 / Cycle 1	First Dose of TV combinations, Safety Labs
Interim	Every 3 Weeks	Treatment infusion (Day 1 of each 21-day cycle), Safety Labs, Efficacy Assessment (Tumor imaging every 6-9 weeks)
End of Study	Follow-up	End of Treatment scan, 30-day safety follow-up, Long-term survival tracking

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Definition:** Incidence of Dose-Limiting Toxicities (DLTs) to establish the MTD and RP2D (Phase 1b); Objective Response Rate (ORR) per RECIST v1.1 defined as the proportion of patients with confirmed complete or partial response (Phase 2). * **Measurement Method:** Investigator assessment of radiological imaging.

7.2 Secondary & Exploratory Endpoints * Number of adverse events (AEs). * Objective Response Rate (ORR) per RECIST v1.1 for the dose escalation phase. * Duration of Response (DOR) per RECIST v1.1 and Time to Response (TTR). * Progression-Free Survival (PFS) and Overall Survival (OS).

8. Safety & Adverse Event Reporting

- Adverse Event (AE) Monitoring:** Routine laboratory evaluations and clinical assessments monitoring the total number of adverse events (AEs). Key monitored AEs included ocular toxicities, anemia, diarrhea, nausea, and thrombocytopenia (Grade ≥ 3).
- Serious Adverse Events (SAEs):** Standard oncology reporting within 24 hours to the sponsor and regulatory authorities.
- Data Monitoring Committee (DMC):** Independent safety review committee to evaluate dose-escalation DLTs and clear transition to the dose-expansion phase.

9. Statistical Analysis Plan (SAP) Summary

- Analysis Populations:** Safety Set (all patients receiving ≥ 1 dose of study treatment); Full Analysis Set (efficacy population excluding patients with protocol violations).
- Sample Size Justification:** Sample sized to provide adequate precision for estimating ORR using exact Clopper-Pearson two-sided 95% Confidence Intervals (CIs).

- **Handling of Missing Data:** Time-to-event outcomes (DOR, TTR, PFS, OS) were analyzed utilizing Kaplan-Meier methodologies.
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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) guidelines and the Declaration of Helsinki.
 - **Monitoring Plan:** Routine clinical site monitoring, centralized data review, and source document verification by the Sponsor.
 - **Audit Trail:** eCRF entry tracking, database locks upon cohort completion, and protocol deviation logging.
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11. Study Identifiers & Governance

- **Primary ID:** {{19-092}}
 - **Registry Number:** {{NCT03786081}}
 - **Posting ID:** {{35140835394056246}}
 - **Trial Status:** {{ACTIVE_NOT_RECRUITING}}
 - **Sponsor/Lead Organization:** {{Pfizer}}
 - **Study Title:** {{Safety and Efficacy of Tisotumab Vedotin Monotherapy & in Combination With Other Cancer Agents in Subjects With Cervical Cancer (innovaTV 205 / GOG-3024 / ENGOT-cx8)}}
 - **Official Regulatory Title:** {{A Phase 1b/2 Open-Label Trial of Tisotumab Vedotin (HuMax®-TF-ADC) Monotherapy and in Combination With Other Agents in Subjects With Recurrent or Stage IVB Cervical Cancer}}
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12. Clinical Framework (Cervical Cancer Specific)

- **Primary Condition:** {{Cervical Cancer (Recurrent or Stage IVB)}}
 - **Diagnosis Threshold:** {{Histologically confirmed squamous, adenosquamous, or adenocarcinoma of the cervix, unamenable to curative therapy}}
 - **Medical Methodology:** {{Antibody-Drug Conjugate (ADC) combined with immune checkpoint inhibitors, anti-angiogenics, or platinum chemotherapy}}
 - **Optimization Target:** {{Objective tumor response (ORR) and sustained disease control}}
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13. Study Procedures & Methodology

- **Management Pathway:** {{Standard oncology evaluation per RECIST v1.1 criteria}}

- **Education Protocol (HCP):** {{Site initiation visits focused on ADC administration and ocular toxicity mitigation}}
- **Education Protocol (Patient):** {{Informed consent processing and specific counseling on potential adverse events (e.g., eye care protocol)}}
- **Quality Audit Mechanism:** {{Centralized image reading and routine clinical monitoring visits}}

14. Observation & Follow-Up Schedule

- **Total Duration:** {{Follow-up until disease progression, death, or study termination}}
- **Onsite Visit Cadence:** {{Day 1 of every 21-day cycle (Q3W) or alternative 28-day cycle for specific expansion arms}}
- **Remote Monitoring Frequency:** {{As medically indicated between cycles}}
- **Checklist Review Interval:** {{Prior to each infusion cycle for safety and compliance}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				